

Tenax Therapeutics, Inc. (NASDAQ: TENX)

R / f(M) Diligence Report · prepared 2026-06-04 · price ~\$10.77 · CIK 0000034956 · one asset, one binary

VERDICT — ELEVATE (disclosure-honesty) · fairly priced (~30% implied PoS ≈ honesty-adjusted) · dilution-heavy. No long edge, no clean short. A cash-funded, fixed-date binary (Phase 3 LEVEL, 6MWD primary, topline Q3 2026) whose *trial story is marketed more bullishly than its own Phase 2 evidence supports*. Not a distress/solvency case — the balance sheet is honest and strong. An initial "trades below cash" read was a *stale-share-count artifact* and is retired below (F-5).

The catalyst

Item	Detail
Asset	TNX-103 (oral levosimendan), K-ATP activator / calcium sensitizer, in PH-HFpEF
Pivotal trial	LEVEL — NCT05983250 , Phase 3, n=230, all-comers PH-HFpEF (US/Canada)
Primary endpoint	Change in 6-minute walk distance (6MWD) at Week 26; 25 m effect; powered >90% (BSSR)
Readout	Topline Q3 2026 (CT.gov primary-completion Oct-2026). Fixed-duration trial → calendar-fixed, not event-driven
Other trials (do not conflate)	LEVEL-2 (NCT07288398, n=540, readout 2028); long-term OLE (NCT07436689)
Cash	\$118.8M (3/31/26), runway to ≥Q1 2028 — funded well past the readout

Findings

F-1 Endpoint switch after a missed Phase 2 primary **ELEVATE**

R (claim & source): Tenax markets the Phase 2 **HELP** trial as “Positive Results ... which formed the basis for the Phase 3.”[BusinessWire, 2020-06-02; *Circ: Heart Failure* publication, 2021]

f (method): Compare the Phase 2 *primary* endpoint result against the Phase 3 *primary* endpoint.

M (authority): ClinicalTrials.gov NCT03541603 (HELP primary = “Change in PCWP with bicycle exercise”) vs NCT05983250 (LEVEL primary = 6MWD).

Finding: HELP **MISSED its primary endpoint** (exercise PCWP −1.4 mmHg, 95% CI −7.8 to 4.8, **p=0.65**). The 6MWD result that is now the LEVEL *primary* was a HELP **secondary** (+29.3 m, p=0.033) in **n=37**, responder-enriched. Marketing a primary miss as “positive” and slotting the Phase 3 primary from the surviving secondary is the classic endpoint-substitution pattern — it materially elevates the risk of a Phase 3 miss.

F-2 Responder enrichment dropped (HELP → LEVEL) **ELEVATE**

R (claim & source): LEVEL is positioned as a registrational confirmation of HELP’s 6MWD signal.

f (method): Compare HELP’s enrollment design to LEVEL’s eligibility (CT.gov).

M (authority): NCT03541603 : open-label **lead-in dose phase**; only acute responders (≥4 mmHg acute PCWP drop, 37/44 = 84%) randomized. NCT05983250 : **no responder lead-in** — all-comers PH-HFpEF.

Finding: HELP enriched for acute responders; LEVEL is unselected. A marginal 6MWD signal observed in 37 enriched patients must now replicate in 230 all-comers — a well-documented failure mode for functional endpoints.

F-3 “Novel, first-in-class” on a 25-year-old molecule REVIEW

R (claim & source): “Novel, first-in-class K-ATP channel activator / calcium sensitizer.”

f (method): Check the molecule’s regulatory/commercial history.

M (authority): Levosimendan (Simdax, Orion Pharma) — IV calcium sensitizer marketed in Europe since ~2000 for acute decompensated heart failure.

Finding: Spin, not a lie: “first-in-class” applies only to the *oral formulation in this indication*. The framing oversells a repurposed legacy molecule — a microstructure tell of optimistic positioning.

F-4 Cash & runway — VERIFIED in the filing CLEAN

R (claim & source): “\$118.8M cash; runway through at least Q1 2028.”

f (method): Balance-sheet check against the 10-Q itself (not just the press release).

M (authority): 10-Q filed 2026-05-12, accession 0001193125-26-219383 : verbatim “\$118.8 million” and “through at least the first quarter of 2028.”

Finding: VERIFIED. Funded past the Q3-2026 readout. The 1-for-80 reverse split (Jan-2024) and dilution history are **disclosed and priced** — re-flagging them is not a signal.

F-5 Cap structure: “sub-cash” RETIRED + dilution finding CORRECTION

R (claim & source): An initial read (on the cover/stale share count of 9.31M) suggested TENX traded *below* net cash (~\$12.75/sh) — implying a market PoS of only ~6–13%.

f (method): Pull the full cap structure from the 10-Q equity notes (cover count is stale/partial): current common + pre-funded warrants + warrant/option overhang; use point-in-time rollforward tables, not the weighted-average EPS line.

M (authority): 10-Q equity notes: common **24,275,500** (3/31/26); **pre-funded warrants 24,566,607 @ \$0.01** (≈ shares, don’t expire); Aug-2024 warrants **10,232,295 @ \$4.50** (expire 30 trading days after LEVEL topline); Feb-2024 0.22M @ \$5.65; options 7.87M @ \$7.34.

Finding: Economic base = **48.84M** (not 9.31M). Net cash ≈ **\$2.43/sh** → stock at ~**3–4.5x cash**, NOT sub-cash — the bargain thesis is **retired**. Real finding is the opposite: common **2.6x’d in one quarter** (a \$6.04 offering + pre-funded exercises); ~67M fully diluted; the \$4.50 Aug-2024 warrants are **catalyst-wired** (exercise on a win → dilution that caps upside; expire on a loss).

F-6 6MWD endpoint & “powered >90%” framing REVIEW

R (claim & source): A prespecified Blinded Sample-Size Re-estimation (BSSR) shows LEVEL “powered well over 90%.” [PR 2025-12-17]

f (method): Identify what a blinded BSSR actually measures.

M (authority): BSSR is **blinded** — it confirms the pooled *variance* assumption, not the treatment effect.

Finding: “Powered” ≠ “likely to succeed”: the >90% holds only if the true all-comers effect is the assumed 25 m (F-2 says it likely regresses below). 6MWD is effort-dependent/placebo-prone and the 25 m bar is modest; FDA has been mixed on 6MWD-only in PH/HF.

Valuation & PoS framing

- **Fixed-date binary, not event-driven.** LEVEL reads 6MWD at a fixed Week 26 (last-patient-in ~Q1-2026), so the readout is calendar-fixed (~Q3–Q4 2026) regardless of outcome — *no* hazard-slip / timing↔outcome coupling (unlike an event-driven survival trial). The binary is purely the outcome.
- **Market-implied PoS ~22–37% (center ~30%)** — from price = $p \cdot U + (1-p) \cdot D$, with failure floor $D \approx$ economic cash/share $\sim \$2.4$ and win-state $U \approx \$25\text{--}40$ (first approvable PH-HFpEF therapy). This sits *inside* the honesty-tells-adjusted estimate of ~25–35% (real but marginal 6MWD signal + adequate powering, minus the failed Phase-2 primary, dropped enrichment, and the PH-HFpEF graveyard). **Roughly fair.**
- **Model U on the fully-diluted ~67M**, not the 24.3M common — a win triggers the \$4.50-warrant exercise window (30 trading days post-topline), diluting the win-state.
- **PoS is unidentifiable** — quote the range, not a point.

Bottom line

The framework correctly flags LEVEL as **over-marketed** (failed Phase-2 primary sold as “positive,” endpoint switched to a marginal enriched-population secondary, enrichment then dropped) → **ELEVATE**, do not take the headline PoS. But the market has already priced this — TENX trades at a normal ~3–4.5× cash with an implied PoS (~30%) in line with the honesty-adjusted estimate, *and* carries heavy dilution/overhang. So: **no short-side honesty-alpha** (the “lie” is detected and already in the price) and **no contrarian long** unless you hold a science view that the all-comers 6MWD replicates against the disclosed evidence. The cap-structure step earned its keep — it killed a false “sub-cash bargain” before it became a thesis.

Authorities cited (M)

ClinicalTrials.gov: NCT05983250 (LEVEL), NCT03541603 (HELP), NCT07288398 (LEVEL-2), NCT07436689 (OLE) · SEC EDGAR CIK 0000034956: 10-Q acc. 0001193125-26-219383 (filed 2026-05-12) · Tenax/GlobeNewswire Q1-2026 corporate update (2026-05-12); BSSR PR (2025-12-17); HELP Phase-2 results (BusinessWire 2020-06-02); HELP publication “Levosimendan Improves Hemodynamics and Exercise Tolerance in PH-HFpEF” (2021). Price: IBKR last ~\$10.77.

Methodology note: every finding shows R (claim+source), f (verification method), M (independent authority + URL/identifier), and the adjudicated Finding. UNVERIFIABLE ≠ clean. PoS quoted as a range. Prepared from live SEC/CT.gov/market data with a 2026-06-04 cutoff.